

CAREMETRIC BREATHE

Customer Service Manual

A practical guide to patient history, resupply outreach, insurance orders, and pricing-review handoffs.



AUDIENCE

Customer service representatives,
fulfillment, and resupply coordinators.

EDITION

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About this manual

This guide covers everyday customer-service work in CareMetric Breathe, including the resupply calendar, order history, pricing reviews, and delivery holds. Use the chapter links during a call. Company policy and the patient's verified records guide each decision.

Read the current screen. Your role and your organization's enabled services determine which actions appear. Cost verification, exception approval, publishing, and owner reporting require separate access. Ask your lead when an action is unavailable; never share another person's login. Illustrations are examples, not live records or promises about timing. Follow the current steps and labels in this edition.

Protect patient information. Use approved work systems for patient records, conversations, and exports. Keep private costs and approval notes out of the patient-facing order note. Sign out on shared workstations.

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Platform overview

1.1 · The four jobs CareMetric Breathe does

CareMetric Breathe is one product with four tightly-integrated jobs. Knowing which "job" a customer is in is the single fastest way to solve their problem.

JOB	WHO USES IT	WHAT IT DOES	WHERE IT LIVES
Mask fitter	New patients	On-device facial measurement & clinical questionnaire produce a ranked mask recommendation.	<code>/how-it-works</code> → <code>/capture</code> → <code>/measure</code> → <code>/questionnaire</code> → <code>/results</code>
Product selection & order requests	Patients choosing or replacing supplies	Review products and request an insurance order. Staff verify the request before fulfillment.	Storefront and <code>/admin/fitter/orders</code>
Insurance pipeline	Insured patients	Qualifier form captures insurance + Rx info; leads route to the admin team for verification.	<code>/insurance</code> → admin Insurance Leads
Resupply automation	Existing patients on a replacement schedule	Scheduled reminders and staff-requested email, SMS, and automated calls, with replies available to the team.	admin Conversations / Resupply calendar / Episodes

All four jobs ship as **one customer-facing site** (your storefront domain) and **one internal admin console** mounted at `/admin/*`. There is no separate dashboard URL to remember and no second SPA to load — whatever a customer sees, you can see (and act on) from the same domain.

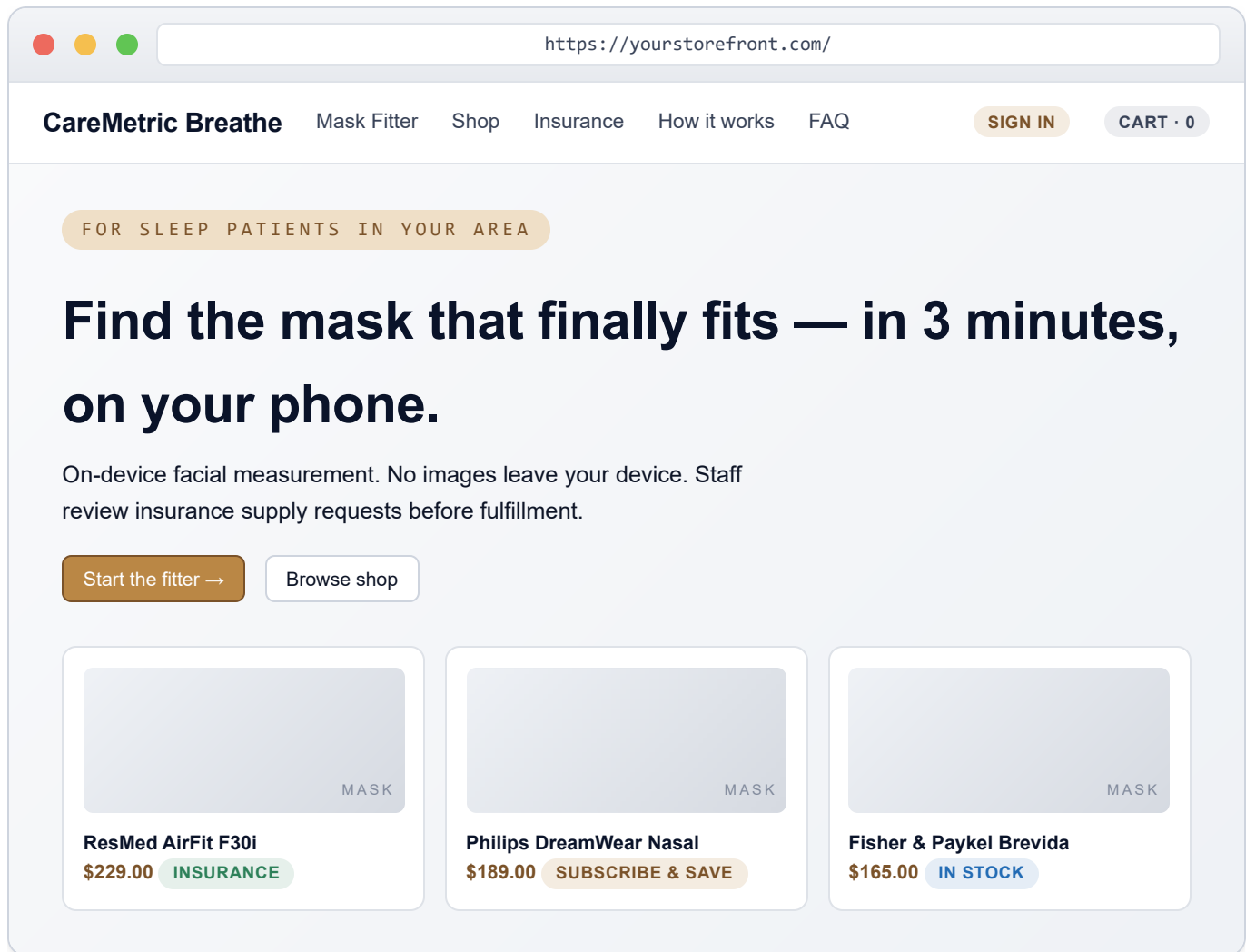


Figure 1 · The customer-facing storefront. Mask fitter, shop, and insurance qualifier all live at the same URL.

1.2 · What to explain to a patient

- **Mask fitting:** on-device measurements help recommend a mask; a clinician must resolve clinical concerns.
- **Repeat supplies:** staff can check recorded items, quantities, order dates, and replacement rules before preparing a request.
- **Contact:** reminders ask whether supplies are needed. A calendar date or unanswered reminder is not permission to ship.
- **Insurance:** coverage, prescription validity, quantities, and required authorizations need verification. A replacement-rule result alone does not guarantee payment.
- **Secure signatures:** the patient reviews the proposed items and paperwork. The link does not charge a card. Do not promise shipment before fulfillment is cleared.

1.3 · Privacy & the on-device promise

Two non-negotiable rules govern everything we do with patient data — repeat them to any patient who asks how we keep them safe.

- **Camera images and video frames never leave the browser.** Only numeric facial measurements are transmitted. No image is ever logged, persisted, or visible to staff.
- **Order request bodies are not logged.** Treat every log line as world-readable; PHI lives only in the application database, behind row-level access controls.

You will be asked. Patients ask "where do my photos go?" on roughly one in eight calls. The answer is: nowhere. The scan happens entirely on their device using a public on-device machine-learning model (MediaPipe Face Mesh). The 16 numeric measurements we keep — bridge width, philtrum depth, etc. — are the only thing transmitted to us, and they are encrypted in transit.

The customer journey

2.1 · Storefront & mask fitter

Patients arrive at **your storefront domain** from search, sleep-study referrals, and our doctor partner network. The home page gives them three doors: **start the fitter**, **browse the shop**, or **check insurance**.

The fitter is a five-step flow. Customers can pause and resume; their session is anonymous until they create an account.

- 1 **Consent** (`/consent`). Plain-language overview of what we measure and what we do not store. Patients must explicitly accept before the camera turns on.
- 2 **Capture** (`/capture`). Front-camera live preview with on-screen alignment guides. MediaPipe Face Mesh runs locally; the page only proceeds once landmarks are stable for two consecutive seconds.
- 3 **Measure** (`/measure`). The 16 numeric measurements are computed on-device and sent to the API as a JSON payload. The image is discarded.
- 4 **Questionnaire** (`/questionnaire`). Eight clinical questions: side sleeper, mouth-breather, claustrophobia, facial hair, glasses, etc. Each maps to mask attributes in the catalog.
- 5 **Results** (`/results`). Top three masks are shown, ranked by a weighted scoring formula that combines fit (measurements) and feel (questionnaire). Patients see a confidence band, not a single "answer".

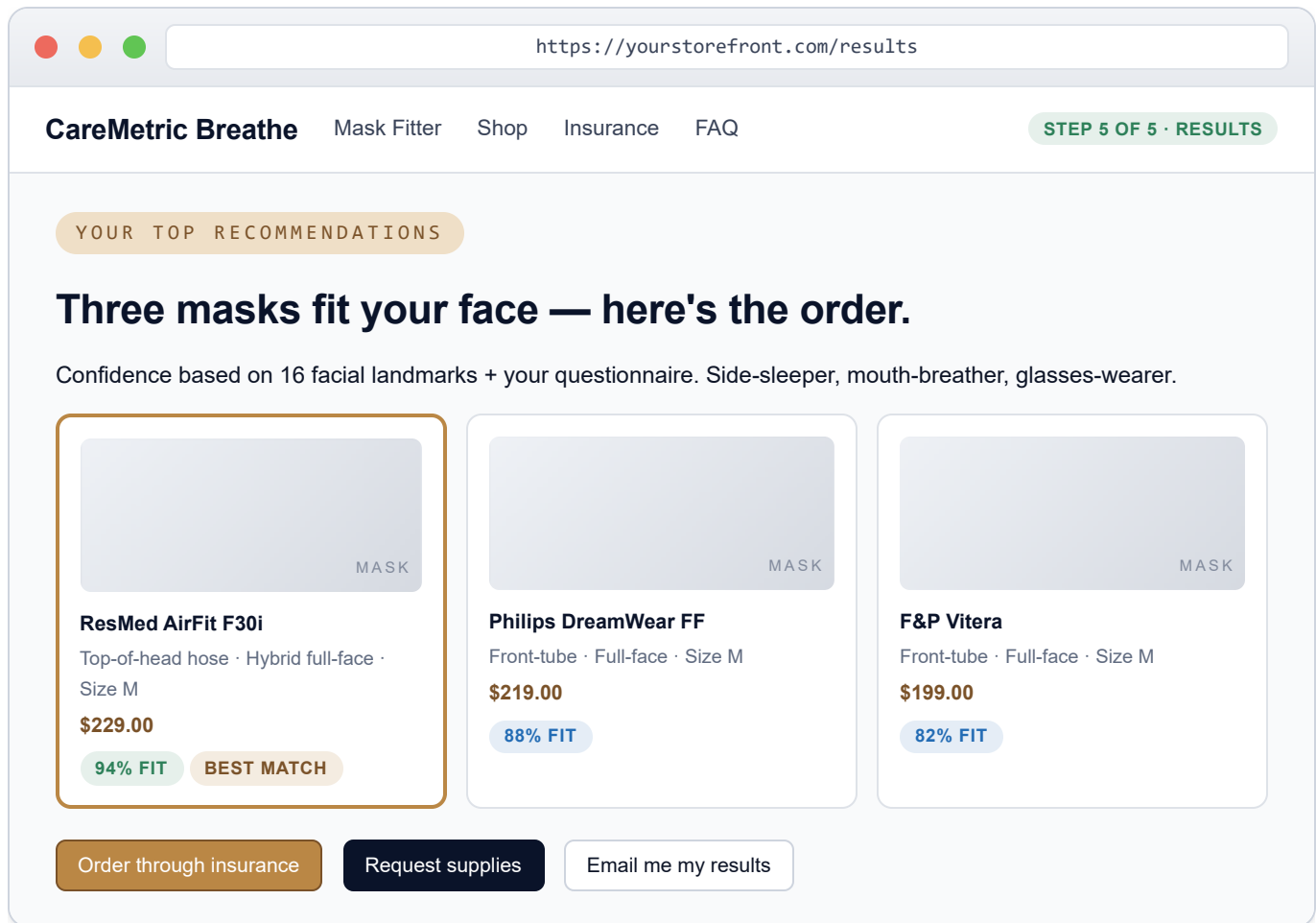


Figure 2 · Illustrative mask results. Use the current insurance-request flow; retired card checkout is not a payment option.

2.2 · Insurance requests & internal self-pay scenarios

The storefront insurance form at </insurance> collects information for staff review. Verify the correct patient, prescription, coverage, and requested quantities in the patient record. Obtain missing evidence before promising an order or patient amount.

No patient card checkout. Patient card checkout and recurring shop billing are retired. *Internal self-pay* in Pricing & Profitability is a staff planning scenario. It neither charges a patient nor establishes an approved payment route. An insurance signature link also collects no payment. Refer payment and coverage questions to authorized billing staff.

The billed amount and expected collections are different. A pricing target cannot increase a contracted insurance payment or create patient liability. Explain only amounts verified by billing.

2.3 · Returning patients & repeat requests

- 1 Confirm identity and open **Patients** → the patient → **Resupply**.

- 2 Read **Order history**: exact item, quantity, status, and shipping dates. Ask whether the patient still uses and needs those supplies.
- 3 Check **Supplies & next eligibility**, the prescription, current address, and contact preference. Resolve unknown or restricted eligibility before fulfillment.
- 4 Prepare a new insurance signature request or use the patient's resupply draft. Old history is a reference; the former item, price, or delivery evidence may no longer be valid.

See [patient history](#) and [signature orders](#) for detailed steps.

The admin console

Sign in at `/admin/sign-in`. The console shows actions allowed for your role and enabled for your organization. CSR access can include patients, conversations, order requests, and pricing evaluation. Authoritative cost verification, exception approval, publishing, and owner reporting are management responsibilities. Ask your lead about missing or disabled actions.

3.1 · Dashboard & live KPIs

Sign in at `/admin/sign-in` and you land on the dashboard. Six KPI tiles drive every other page on the site — each is a clickable link to its filtered queue.

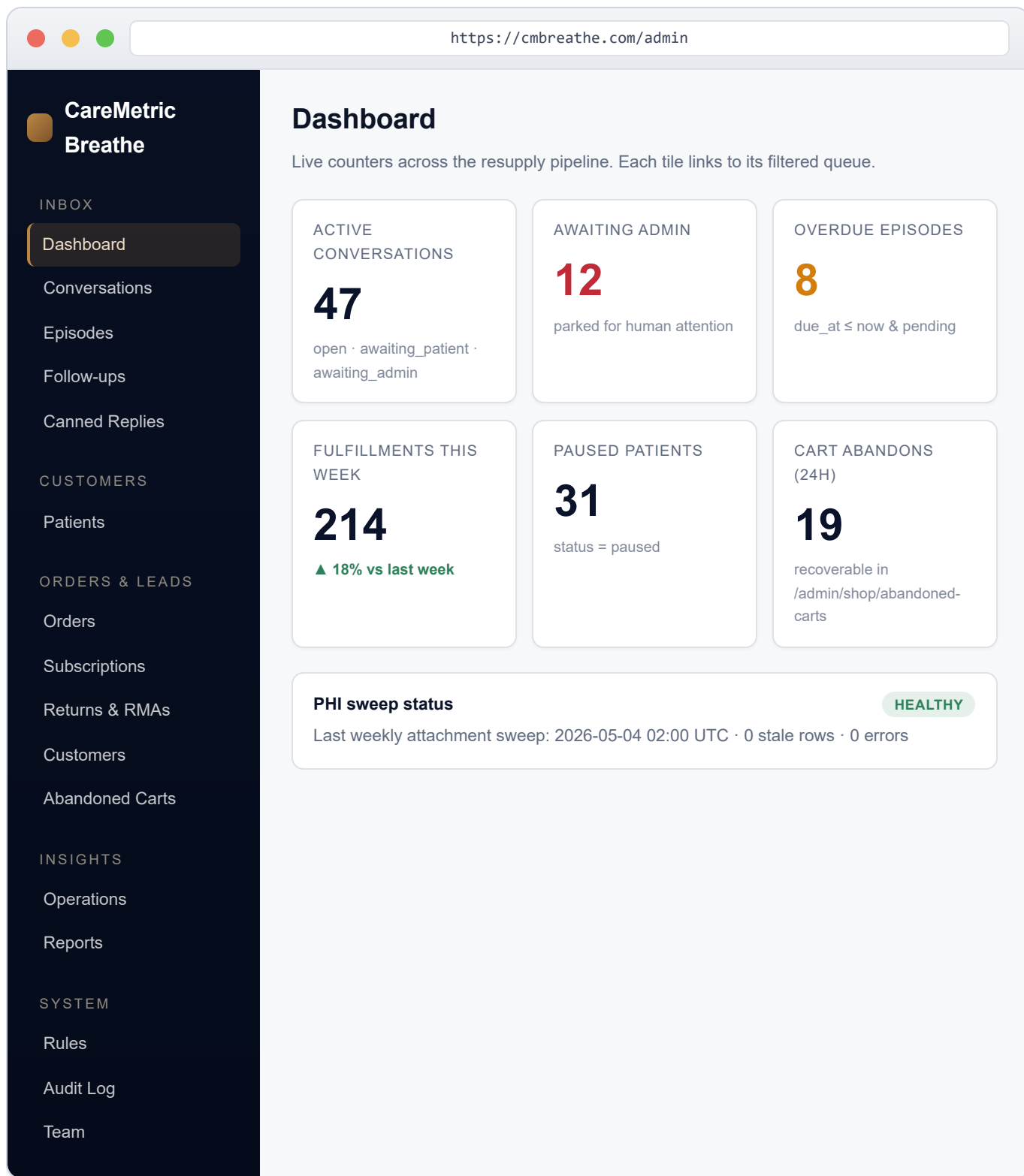


Figure 3 · The admin Dashboard. Click any KPI to land on its filtered queue.

3.2 · Conversations (SMS, email, voice)

The Conversations page is your inbox. Every channel — outbound SMS, inbound replies, MMS attachments, email replies, and voice transcripts — funnels into the same threaded view, keyed to the patient. Use the status filter to focus:

- **AWAITING_ADMIN** — patient replied; we owe them a human response. Default queue.
- **AWAITING_PATIENT** — we sent something; waiting on them. Don't bug them again.
- **RESOLVED** — closed; archive view.
- **SNOOZED** — comes back to *awaiting_admin* after the snooze interval.

https://cmbreathe.com/admin/conversations?status=awaiting_admin

Conversations

12 awaiting admin · 47 active · 318 resolved this week

Search by phone, name, episode... Status: Awaiting admin Channel: All Assignee: Me + New conversation

PATIENT	LAST MESSAGE	CHANNEL	EPISODE	STATUS	WAIT
M. Alvarez +1 (267) 555-0119	"Mask leaks at the bridge — can I swap?"	SMS	EP-4012 · Mask	AWAITING ADMIN	12m
D. Patel d.patel@...	"Confirm the F30i is in stock before you ship?"	EMAIL	EP-4007 · Resupply	AWAITING ADMIN	28m
J. Yoo +1 (215) 555-0144	MMS · 2 ATTACHMENTS "this rash okay?"	SMS · MMS	EP-3998 · Cushion	AWAITING ADMIN	42m
R. Chen +1 (484) 555-0102	"Voicemail · 0:38 · transcript ↗"	VOICE	EP-3992 · Headgear	AWAITING ADMIN	1h 04m
K. Brooks +1 (267) 555-0188	"Please pause my resupply reminders for July"	SMS	SUB-2204	AWAITING ADMIN	1h 22m

Figure 4 · Conversations queue, filtered to *awaiting_admin*. Wait time turns amber after 30 minutes and rose after 2 hours.

Click into a conversation and the detail view splits the screen into three columns: **thread** (left), **composer** (center), and **Patient 360** (right). The composer offers an AI-suggested draft you can accept, edit, or discard — drafts are never sent without your click.

CareMetric
Breathe

INBOX

Dashboard

Conversations

Episodes

Follow-ups

https://cmbreathe.com/admin/conversations/CV-7741

M. Alvarez · CV-7741

AWAITING
ADMIN

Snooze

Resolve

SMS · +1 (267) 555-0119 · Episode EP-4012 · Mask resupply

THREAD · 6
MESSAGES

CAREMETRIC
BREATHE · 2D AGO

Time for your monthly resupply, M.! Reply YES to ship the F30i (med).

M. ALVAREZ · 2D
AGO

YES

CAREMETRIC
BREATHE · 2D AGO

Got it — shipping today, tracking by tomorrow. 🌟

M. ALVAREZ · 12M
AGO

Mask leaks at the bridge — can I swap?

AI DRAFT · REVIEW
& SEND

Sorry to hear that, M. — bridge leaks are usually a sizing issue. We'll send a small replacement at no charge under our Comfort Guarantee. Want it shipped to your home address on file?

COMPOSE

Sorry to hear that, M. — bridge leaks are usually a sizing issue. We'll send a small replacement at no charge under our Comfort Guarantee. Want it shipped to your home address on file?

Attach

Macros

Send
SMS

QUICK ACTIONS

Re-ship same SKU

Open RMA

Review reminder preferences

Escalate to lead

PATIENT 360

Maria Alvarez

Patient since Aug 2025 · 9 orders

Insurance: INDEPENDENCE
BLUE CROSS

Mask: F30i (M) · 30-day cadence

Last fitter run: 2025-08-12

Sleep position: side · mouth-breather: yes

RECENT ORDERS

ORD-9482 · Apr 30 · F30i M · \$0 (insurance) · ✓ delivered

ORD-9311 · Mar 28 · F30i M · \$0 (insurance) · ✓ delivered

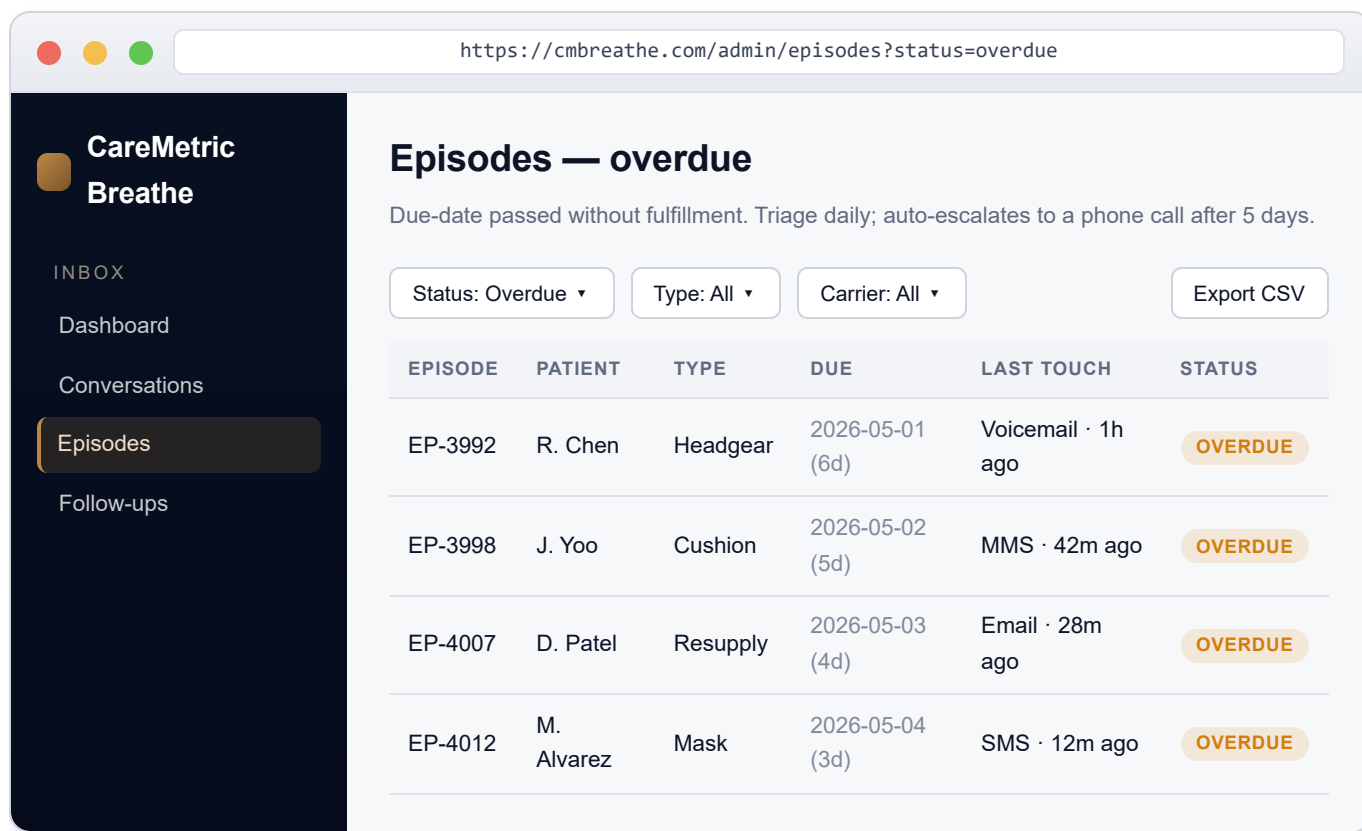
Figure 5 · Conversation detail with AI-drafted reply. The amber-bordered bubble is the suggestion — nothing sends until you click.

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3.3 · Episodes & resupply automation

An **episode** is the unit of work the system tracks — one mask resupply, one cushion replacement, one headgear swap. The rule engine (`/admin/rules`) creates episodes automatically based on each patient's replacement schedule. You'll usually see them as the "Episode" link in the conversation list.

Use the current episode filters: **Outreach**, **Awaiting reply**, **Address hold**, **Confirmed**, **Fulfilled**, **Declined**, **Expired**, and **Canceled**. **Overdue** identifies late open work. A confirmed response is not a recorded shipment; resolve prescription, order, and delivery requirements before fulfillment.



The screenshot shows the CareMetric Breathe interface. The left sidebar has a dark blue background with the logo and navigation links: INBOX, Dashboard, Conversations, Episodes (highlighted), and Follow-ups. The main content area is titled 'Episodes — overdue' and includes a sub-header: 'Due-date passed without fulfillment. Triage daily; auto-escalates to a phone call after 5 days.' Below this are filters for Status (Overdue), Type (All), and Carrier (All), along with an 'Export CSV' button. The table below lists four overdue episodes.

EPISODE	PATIENT	TYPE	DUE	LAST TOUCH	STATUS
EP-3992	R. Chen	Headgear	2026-05-01 (6d)	Voicemail · 1h ago	OVERDUE
EP-3998	J. Yoo	Cushion	2026-05-02 (5d)	MMS · 42m ago	OVERDUE
EP-4007	D. Patel	Resupply	2026-05-03 (4d)	Email · 28m ago	OVERDUE
EP-4012	M. Alvarez	Mask	2026-05-04 (3d)	SMS · 12m ago	OVERDUE

Figure 6 · Illustrative overdue episodes. Actual timing follows your organization's configuration. Check recent conversations before another contact.

3.4 · Patient history & replacement eligibility

Open **Patients**, find the correct person, and choose **Resupply**. From the calendar, **Orders & eligibility** opens the same summary without leaving the list.

- 1 What was ordered:** each **Order history** row is a recorded supply order line, not necessarily a whole order. Match item name/SKU, **Qty**, **Ordered**, and **Status / reference**. Use the recorded quantity, not an assumed standard pack size.
- 2 When it moved:** read **Shipped** and **Delivered** separately. A queued line, signature, or missing date does not prove shipment or receipt. Use the reference for fulfillment follow-up.

- 3 **Older records:** use **Older** and **Newer** through 25 lines at a time. The total counts lines. If a page fails, retry it or use **Newer**; a failed page is not empty history.
- 4 **Signature requests:** when present, **CSR orders & signatures** shows items, quantities, creation date, and signature status linked to the patient's most recent 50 resupply drafts. A signed request may also appear in fulfillment history; do not count it as a second shipment.
- 5 **Replacement checks:** read every **Supplies & next eligibility** row: supply/SKU/HCP/PCS mapping, prescription expiry, **Last ordered**, **Next eligibility**, and **Scheduled resupply**.

WHAT YOU SEE	HOW TO USE IT
Eligible by replacement rule	Recorded history passes the mapped rule. Check available units, valid prescription, insurance coverage, and patient need before fulfillment.
Interval opens... / Quantity limit reached	The interval or quantity allowance still restricts the request. Read the reason, units available, and any Next quantity review date. An interval date does not mean unlimited supply.
Needs eligibility review	The app cannot establish a mapped rule, for example for an unmapped item. Ask eligibility or billing staff to review it. Unknown is not eligible.
Scheduled resupply / Not scheduled	Scheduling controls review/contact timing. It is separate from replacement eligibility and coverage. No active prescription requires review or renewal before scheduling.

Useful wording: “Your record shows this item and quantity on this date. I’ll check when another supply is allowed and confirm that you need it before we prepare the request.”

3.5 · Follow-ups, canned replies, macros

Three small but heavily-used tools:

- **Follow-ups** (`/admin/followups`) — your personal "come back to this" queue. Snooze a conversation and it lands here at the chosen time. Also surfaces every conversation assigned to you that has been silent for 24 hours.
- **Canned Replies / Macros** (`/admin/macros`) — shared, branded message templates with variable interpolation (`{{first_name}}` , `{{mask_model}}` , `{{rma_link}}`). Use these instead of free-typing common responses.
- **Rule Tester** (`/admin/rule-tester`) — sandbox for the rule engine. Operators (not agents) use it to dry-run a rule against a fake patient before flipping it on.

3.6 · Resupply calendar & individual or bulk outreach

Open **Resupply calendar** at `/admin/resupply-calendar` , or **Open resupply calendar** from a patient's summary. The calendar groups scheduled supply cycles by patient. A person may have several supplies or due dates; outreach selection is by patient.

- 1 Choose **Month calendar** for planning or **Due now & overdue** for current work. Navigate with **Previous month**, **Next month**, or **Today**. Select a day to filter the list; **Show whole month** clears that day filter.
- 2 Use **Search patients or supplies...** to narrow the list. Read patient and supply-cycle counts, due dates, contact details, and channel preference. Dates use the time zone named on the calendar, separate from UTC owner-report dates.
- 3 Choose **Orders & eligibility** to inspect history and limits. Check recent conversations, prescription, active status, and permission to contact. Future calendar entries are for planning; the send check skips supplies not yet scheduled due.
- 4 For one patient, select their checkbox or use outreach controls in their Resupply summary. For a batch, select individual patients or **Select visible patients (up to 50)**. Larger lists require separate reviewed batches. Changing month, day, view, or search clears selection; check it again.
- 5 Choose **Email**, **SMS**, or **Automated call**. Automated call queues a system call; it does not connect a live CSR call. For a staff call, follow the approved calling workflow and record the conversation.
- 6 Review the confirmation's names, count, and channel. It asks whether the patients still use their supplies and need a refill. Choose **Queue outreach** only when the list is correct, or cancel to revise it.
- 7 Read each result and the **queued · skipped · failed** totals. Open **Conversations** for delivery information and replies. A queue result does not create an order or show patient agreement.

OUTREACH RESULTS AND CONTACT LIMITS

RESULT	NEXT STEP
Queued	Accepted for processing. Do not requeue these patients or switch channels just to obtain another confirmation. Workers recheck restrictions before sending. Sent/accepted is not confirmed delivery; a queued call is not an answered call.
Skipped	Read the reason: recent contact or queued work, not yet due, inactive patient, prescription review, missing email/phone, or contact hours. Correct only verified issues. SMS and calls respect local contact hours. Do not override opt-outs or change preferences merely to send.
Failed	Review and correct the problem, then select only unresolved patients and use the channel action again. The current view excludes successfully queued recipients; do not rely on that after changing views. Do not resend the entire batch.
Request error / uncertain result	Check Conversations and the current queue state before retrying. A lost response may follow a successful send. Duplicate-contact protection may retain the earlier attempt; ask your lead to investigate rather than repeatedly sending or switching channels.

Channel availability and contact limits. Contact information does not establish consent or a working delivery service. Recent conversations within 48 hours can prevent another resupply contact across channels. Unavailable email, SMS, or calling services need an administrator; do not use a personal account. Review opt-outs, contact hours, and replies before proceeding.

Insurance orders & pricing reviews

4.1 · Create a signature order

Open **Orders** at `/admin/fitter/orders`. In **Signature orders**, choose **Create order**. This is an insurance request and paperwork workflow. Nothing is charged on the patient's signature link.

- 1 Confirm the patient and enter their name and current email or mobile number. At least one contact method is needed. In **Internal profitability review**, use **Patient for pricing review** to select the actual record rather than relying on a typed customer name.
- 2 Add exact descriptions, quantities, and billed unit amounts. Use **Add item** where needed. Patient signature orders support up to 20 lines and 1–99 units per line; larger internal simulations cannot attach.
- 3 Use **Evaluate items** or **Choose approved review** as described below. When reviews are required, the order cannot proceed without a valid approved insurance review. When optional, an order without one remains unreviewed.
- 4 Select applicable **Standalone paperwork to sign**. Use **Document packets** for onboarding or custom patient packets. Confirm the paperwork and items belong to this request.
- 5 If needed, add a **Note to customer**. The patient sees it; exclude supplier costs, margins, internal approval reasons, and staff-only notes.
- 6 Review the complete order, then choose **Create & send** once. Keep its reference and read the communication result. A created request is not a signature, payment, or shipment.

4.2 · Evaluate costs, collections & delivery

Use **Evaluate items** on the order, or open **Pricing & Profitability** at `/admin/pricing` → **Item review**. CSRs can evaluate permitted scenarios and request review; entering an estimate does not make supplier or insurance evidence verified.

- 1 Choose the insurance scenario and correct patient. For each line select the exact catalog item and supplier offer, quantity, billed unit amount, and stock or dropship fulfillment. Regular **Supplier costs** offers record cost per canonical selling unit, with units per pack recorded separately. Do not divide an already normalized unit cost by pack size again. The provisional **New items** comparison below instead starts from purchase-pack cost.

- 2 Review acquisition, inbound freight, dropship charges, outbound delivery, handling, packaging, other fees, and applicable reserves. Do not double-count a charge included in another cost. Blank is unknown; use zero only when evidence establishes zero.
- 3 Check delivery country, postal code, and supplier service against the saved patient address and supplier coverage. For stock shipping, enter measured weights and dimensions for every parcel, choose **Get shipping rates**, and select the appropriate returned rate. Missing configuration or a missing parcel is not free delivery.
- 4 For insurance, select the **Verified collection profile** matching this patient and exact items/quantities. An unverified **Expected total collections (\$)** entry remains an estimate. Ask management or billing for current source evidence. Billed amounts do not establish what insurance or the patient will pay.
- 5 Review applicable processing fees, their tax-inclusive charge basis where configured, reserves, discounts, and allocated overhead. Shipping charged in an internal self-pay scenario differs from actual delivery cost. Do not assume card fees apply to insurance collections.
- 6 Check **Review valid through (UTC)** and evidence dates. Carrier estimates last no longer than their returned expiry, currently up to 30 minutes; a rate is not a purchased label or guaranteed carrier price.
- 7 Choose **Evaluate profitability**. Read delivered cost, profit after variable costs, policy margin, the rule basis, and every issue. Changes to patient, items, supplier, quantities, address, or assumptions require evaluation again.

RESULT	MEANING AND NEXT ACTION
Meets target	The calculation meets policy. A complete, verified scenario can save as approved; inspect saved approval status and expiry before use.
Approval needed	A permitted exception, such as a below-target result or estimated evidence, needs explicit manager review. Use Request manager approval ; requesting does not approve it.
Cost information needed	Missing or stale evidence prevents a complete review. Obtain the information and reevaluate. Saving a draft does not clear the issue.
Blocked	A hard floor, minimum contribution, or other policy limit prevents approval. A manager's exception cannot bypass the hard floor. Correct assumptions or arrange an authorized alternative.

Margin is not markup. The company chooses its target and hard floor; there is no universal recommended percentage. Check whether policy includes allocated overhead. Review economics are not proof of cash collected or total company profit.

4.3 · Save, request approval & attach the exact review

- 1 Use the displayed action: **Save review**, **Request manager approval**, or **Save draft review**. Give the lead the patient, exact items/quantities, evidence, expiry, and issue to resolve through an approved internal channel.
- 2 Once approved, choose **Use approved review in order**. Alternatively choose **Choose approved review**, inspect the patient's saved approvals, then **Use this review**.
- 3 Confirm the attached patient, items, quantities, billed amounts, and deadline. Approval belongs to that exact order. A line change removes or invalidates the attachment; evaluate again instead of reusing an old approval.
- 4 At **Create & send**, the system rechecks policy, approved revision, supplier/collection evidence, and delivery details. Refresh and prepare a current review after changes or expiry. Repeated submission of an unchanged request is protected against a second committed order.

Once linked to an order, the review is **bound** and cannot authorize another order. Patients see the signed items and appropriate amounts, not private costs, contribution, approval notes, or financial history.

4.4 · New-item proposals & patient batches

New items: when no suitable priced catalog item exists, open **New items** in Pricing & Profitability. Record the proposal and evidence, then choose **Compare provisional supplier costs**. Use **Add supplier option** to compare up to five suppliers for the same requested quantity, including pack price, units per pack, minimum pack purchase, dropship, delivery, and handling.

Read total purchase outlay, purchased/surplus units, availability, expiry, and delivered-cost completeness together. A lower apparent unit cost may require extra packs. Unknown charges stay unknown; estimated or unavailable supply is not a firm offer. Choose **Submit item proposal** for management review. A firm customer price requires canonical item mapping and a verified supplier offer; provisional comparison creates neither.

Patient batch review: where available to your role, select up to 50 open drafts. Up to 200 open drafts are shown at a time. Choose **Evaluate selected drafts**; the batch checks each patient's newest matching exact-item approval and separates ready, stale, missing-review, and unavailable results. It neither contacts patients nor creates orders. Use individual review to correct items/quantities, obtain evidence, or choose another valid approval when the newest one is stale. Use **Show only patients needing attention** to focus the results and **Open resupply order review** for follow-up. This differs from the calendar outreach batch.

4.5 · Signatures, retries & changed delivery details

- 1 Follow the request in **Signature orders**. The patient opens the secure link, reviews the items and paperwork, and signs. A response requesting supplies still needs the appropriate order and signature process.

- 2 For a new link to an unsigned request, use **Resend**. Older links become invalid. Resending a priced order does not extend its approved deadline. After expiry, prepare a fresh review/request and cancel the obsolete request as appropriate.
- 3 A signed request with an approved patient pricing review or linked resupply draft can queue fulfillment. Without the required link it stays signed for staff follow-up. Check exact SKU, quantity, status, and errors; signing does not prove shipment.
- 4 Resolve missing/expired prescriptions and other prerequisites. Use **Retry fulfillment** on the existing signed request when available. Do not create a duplicate order to work around an error.
- 5 A changed saved patient address can place unsubmitted priced fulfillment on a delivery hold. Confirm and save the correct address, then give a manager the order reference and reason. Do not restore an incorrect address merely to clear the hold.

Manager delivery handoff. The manager uses **Review delivery**, verifies current address, supplier/service coverage, and fresh freight evidence, then **Preview delivery profitability**. The manager enters **Delivery approval reason**, reviews any permitted exception, and chooses **Approve delivery and retry fulfillment**. Approval records the reason and exact revised forecast. Original signed items, accepted amounts, and original pricing remain intact. Resolving a general address alert alone does not release a priced order; the hard financial floor still applies. Previously shipped work is historical and needs the appropriate shipping follow-up.

Only a manager may reconfirm previously verified, unchanged fees or reserves with new evidence and expiry. Changed supplier costs or delivery zones require updated supplier offers. This cannot invent a missing cost, change the signed price, or turn an estimate into an invoice.

Warehouse handoff: PacWare preview and CSV use the signed fulfillment's exact SKU and quantity. Held, already submitted, or delivery-review-required priced lines are withheld and counted. Investigate omissions; do not put a held line into an old file manually. Export is a manual handoff, not supplier submission or shipment confirmation.

Insights, system, and audit

5.1 · Operations & delivery failures

Two pages exist specifically for the things that go wrong, so you can find them before the patient calls.

- **Operations** (/admin/operations) — health board: SMS delivery rate, email open/bounce rate, voice answer rate, pg-boss queue depth, last reminder scan, last PHI sweep. If anything is red here, escalate to the on-call lead before promising a delivery time.
- **Delivery Failures** (/admin/delivery-failures) — bounced emails, undeliverable messages, and their recorded failure details. Review the conversation, contact information, and existing delivery status before a permitted retry. Do not switch channels to bypass an opt-out or an uncertain earlier send.

5.2 · Reports & manager handoff

Use the operational reports available to your role to follow response, delivery, and fulfillment work. Each page has its own filters and definitions. A contact, prepared line, signed request, or scheduled due date is not a completed sale. **Owner overview** is a management page; see [5.4](#).

For a handoff, provide the internal patient/order reference, selected period if relevant, exact status or error, evidence checked, and decision needed. Share through approved systems. A downloaded report is a dated snapshot, not a live record.

5.3 · Rules, rule tester, audit logs

Operators (not agents) own these screens; they're documented here so you understand what's driving your inbox.

- **Rules** — every automation in the system: when to send the first resupply nudge, when to escalate to a phone call, when to move a conversation to *awaiting_admin*. Listed with a short summary; tap to expand the conditional.
- **Rule Tester** — paste a fake event, see which rules would fire and what each rule would do. Used before rolling a rule change to production.
- **Audit Log** — every admin action recorded with operator, target, timestamp, and IP. PHI never appears in the log — only the IDs and the action verbs.
- **Settings & Team** — admin-only. Invite, promote, demote, and remove team members; rotate the link-signing HMAC if a leak is suspected.

If a customer asks "did someone change my order?" the audit log can tell you who did what and when. From the order detail, click the timestamp on the activity entry — it deep-links into the audit log filtered to that record. This is the single most powerful feature for resolving "I didn't authorize that" disputes in under a minute.

5.4 · Owner overview, planning models & actuals

Management access only: Owner overview at `/admin/analytics/owner` requires management and financial reporting access. Ask a manager for the operational decision you need; do not request another user's credentials.

- **Period:** last 7, 30, 90, or 365 days, or **Custom dates** and **Apply dates**. Dates are UTC; custom end dates are inclusive and today stops at report time. The prior period has the same elapsed duration.
- **Freshness:** use **Refresh** and check the update time. **Download overview CSV** uses the returned report, not unapplied date edits, and is disabled during refresh. A failed section is unavailable, not zero; the other section can still load.
- **Scope:** current queues include older open work. Claims created in the period show payments recorded to date, not cash received that month. Prepared lines are not shipments. Untracked stock is not zero stock. Scheduled due patients still need eligibility and consent checks.
- **Financial activity:** revenue, refunds, costs, and credits use their economic dates. **Completed-review contribution** is a separate lifetime total for bound reviews with costs and collections explicitly complete. Missing, invalid, or future event dates exclude the affected review from that total. These figures are not company-wide net profit or bank cash.

Owner models: managers choose a saved review in **Owner models**, or use **Use in owner models** after item evaluation. Six tools cover pricing strategies, monthly break-even and profit, cost/freight/collection stress, price versus volume, repeat orders and acquisition, and working capital. Entered costs, demand, collection timing, and other assumptions drive results. The models do not publish prices, approve orders, or charge patients. Refreshing the source clears previous results; exported results identify the evidence used for each calculation.

Reviews & actuals: authorized managers or finance staff record real invoice, freight, fee, collection, refund, and credit events against the order review. Enter **Economic occurrence date and time (UTC)** from the evidence, including historical dates. The form defaults to the current minute and rejects missing, impossible, or future manual dates. Keep the same source reference and event identifier for an unchanged retry. If an edited retry conflicts, refresh history before documenting a correction; do not create a duplicate expense or payment. Costs and collections are marked complete separately with a reason; a missing invoice is never a known zero.

Day-in-the-life playbook

6.1 · Common scenarios & how to solve them

SCENARIO A · “SEND WHAT I HAD LAST TIME”

- 1 Confirm identity, then Patients → Resupply → Order history. Read back exact item, quantity, and date.
- 2 Check current need, replacement limits, prescription, coverage, and address.
- 3 Evaluate or attach a current exact-patient review and prepare the insurance signature request. Explain the next step without promising a charge or shipment.

SCENARIO B · “PLEASE STOP THE REMINDERS”

- 1 Record the request in the conversation and use the authorized contact-preference or pause workflow.
- 2 Review recent contact and queued work with the lead before further outreach. Do not switch channels to bypass the request.
- 3 Explain what was recorded. A resupply pause is not a card-subscription cancellation; patient recurring shop billing is retired.

SCENARIO C · “I NEVER GOT MY ORDER”

- 1 Check the signature request and patient Order history. Distinguish signed, queued, shipped, and delivered records.
- 2 Ask fulfillment to verify order reference, tracking, and correct address. Message **Delivery Failures** is not parcel tracking.
- 3 Follow the company's lost-shipment/return procedure and obtain any required replacement approval. Do not promise a free reship based on a cost threshold from an old example.

SCENARIO D · “MY ADDRESS CHANGED AFTER I SIGNED”

- 1 Confirm and save the correct address through the authorized patient workflow. Identify the signed order and whether it has shipped.
- 2 For unsubmitted priced work on hold, ask a manager for **Review delivery** with current cost and freight evidence.

- 3 Keep signed items and accepted amounts intact. Resume the existing fulfillment only after required review; do not create a duplicate request or bypass the export hold.

SCENARIO E · “SEVERAL PATIENTS FAILED IN MY BATCH”

- 1 Read every result. Leave queued patients alone; skipped reasons are not all retryable failures.
- 2 Check Conversations after uncertain responses. Correct verified contact or prescription issues, then select only unresolved recipients.
- 3 If the channel is unavailable or an earlier attempt may have reached the provider, escalate rather than repeatedly requeueing.

6.2 · Escalation & service level targets

The timings below are training examples, not software guarantees or an approved company SLA. Follow your organization's published coverage hours and escalation policy.

CHANNEL	FIRST RESPONSE	RESOLUTION	ESCALATE AFTER
SMS	15 minutes	4 hours	2 hours waiting on you
Email	2 hours	1 business day	4 hours waiting on you
Voice (voicemail)	30 minutes	Same day	1 hour
Web chat	1 minute	15 minutes	5 minutes
Insurance lead	1 business day	3 business days	2 business days

Escalate to your shift lead or designated clinical, billing, pricing, or fulfillment owner. Include the reference, current status, steps taken, and decision needed. A delayed callback must not lead to duplicate outreach, duplicate financial events, or an unapproved shipment.

6.3 · Glossary & quick-reference card

TERM	WHAT IT MEANS
Episode	One unit of resupply work — mask, cushion, headgear, etc. Created by the rule engine; closed when fulfilled.
Conversation	The threaded view of all messages exchanged with a patient (any channel) tied to one or more episodes.
Patient 360	Right-side panel on conversation/episode pages showing the patient's full history.

TERM	WHAT IT MEANS
Replacement eligibility	Mapped interval and quantity rules checked against recorded orders; not confirmation of coverage.
Scheduled resupply	When a supply cycle is due for review/contact. Separate from replacement entitlement, prescription validity, and patient consent.
Approved / bound review	Approved means authorized within its evidence and deadline. Bound means linked to one exact order and unavailable for another.
Delivery hold	Fulfillment waits for required current-address, delivery, and financial checks.
Contribution	Revenue less costs included in the stated calculation. Read evidence and whether allocated overhead is included.
Macro / Canned Reply	Branded message template with variable interpolation. Always prefer over free-text.
Snooze	Move a conversation out of the queue for a chosen interval; it returns to <i>awaiting_admin</i> when time's up.
PHI	Protected Health Information. Names, phone numbers, prescriptions, insurance IDs. Never leaves admin SSO.
pg-boss / worker	The background-job system that runs reminder scans hourly and PHI sweeps weekly. Lives inside the same API process.

Quick-reference card.

- Patients → Resupply: exact items, quantities, dates, and replacement checks.
- Resupply calendar: select at most 50, confirm names/channel, inspect each result.
- Conversations: verify delivery and replies before retrying uncertain outreach.
- Orders → Signature orders: attach the approved review, then Create & send.
- Changed address, expired evidence, or blocked costs: keep the order and hand off for required review.
- Owner overview/models: management reporting and assumptions, not permission to change prices or collect payments.

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